

Diagnosing stalled MRR

Week 10, Lecture 1 · B2C to \$10k MRR

Autumn 2026 · A 10-Week Growth Manual for Technical Founders

What we'll cover

- Why MRR stalls and how to tell the difference between four causes
- The diagnostic quadrant: leak, ceiling, price, channel
- Reading your own dashboard like a stranger would
- The pivot trigger rules: product vs channel vs price
- Honest assessment: should you keep going?
- Live diagnosis: three student products on the projector

The diagnostic quadrant

Four reasons MRR stalls

Most stalls have one primary cause. Naming it correctly determines the fix.

- **Leak:** users arrive but don't activate, or activate but don't retain. The funnel has a hole.
- **Ceiling:** retention is decent, but the wedge channel is saturated. You can't find more users.
- **Price:** the product retains users, but trial-to-paid conversion is low. The value is there; the price isn't right.
- **Channel:** you have product-market fit with existing users, but CAC is unsustainable or the channel is exhausted.

(Alstromer, 2019; Stripe MRR Guide, 2026)

Leak: the symptoms

A leak is a retention problem presenting as a growth problem.

Diagnostic numbers that point here:

- Activation rate below 25%
- D7 retention below 15%
- D30 retention below 10%
- MRR curve that grows in spurts after campaigns, then flatlines or drops

The fix lane: stop acquisition-side work. Fix the onboarding flow, the activation event, or the first-session value delivery.

Ceiling: the symptoms

A ceiling is a channel problem presenting as a product problem.

Diagnostic numbers that point here:

- D30 retention above 20% (the product works)
- Activation rate above 30% (the onboarding works)
- MRR growth flat for 4+ weeks despite ongoing channel output
- CAC rising as the audience narrows

The fix lane: find the second channel. The wedge channel is saturated. Lenny's Racecar framework names five growth component types; you are missing an engine or lubricant.

(Rachitsky, 2022)

Price: the symptoms

A price problem looks like a retention problem but is a willingness-to-pay problem.

Diagnostic numbers that point here:

- D30 retention above 25%
- Trial-to-paid conversion below 4%
- Qualitative: users call the product "nice to have," not "can't imagine living without it"
- PMF survey score below 40% "very disappointed"

The fix lane: raise price, add a higher tier, or close the activation gap between free and paid value.

(Ellis, 2019; Vohra, 2018)

Channel: the symptoms

A channel problem is sometimes called a "distribution problem."

Diagnostic numbers that point here:

- Retention good. Activation good. Paid conversion good.
- CAC above LTV/3, or rising faster than LTV
- Public output count above 50 with no compounding effect
- Zero organic/referral inbound despite 10 weeks of output

The fix lane: find channel-product fit for a different channel. The current channel ceiling is the constraint, not the product.

(Winters, 2023; Verna, 2025)

The self-audit: reading your dashboard like a stranger

The stranger test

A stranger who looked at your dashboard for 3 minutes would ask:

1. What is the MRR number right now?
2. Is the curve going up, flat, or down?
3. Where does the funnel lose the most people?
4. What happened 4 weeks ago when the curve changed direction?

If you cannot answer all four in under 60 seconds, your dashboard is not readable. Fix the dashboard before the diagnosis.

The self-audit checklist

Work through this before section Friday:

- ☐ MRR for each of the 10 course weeks: written down in one row
- ☐ Activation rate for the past 30 days: one number
- ☐ D7 and D30 retention for the most recent complete cohort: two numbers
- ☐ Funnel conversion rates at each AAARRR stage: four or five numbers
- ☐ The quadrant with the most evidence: one word

The checklist is not about having good numbers. It is about having real ones.

The pivot trigger rules

When to pivot the product

Pivot the product when the evidence is clear and you have tried the other fixes first.

Triggers:

- D30 retention below 10% after three activation-flow improvements
- PMF survey below 20% "very disappointed" across two distinct user segments
- No user reports feeling the product is irreplaceable

Do not pivot the product because you are bored with it or because a competitor launched something new.

(Ries, 2011)

When to pivot the channel

Pivot the channel when the product works but the audience is wrong.

Triggers:

- D30 retention above 20% in a small segment but near zero in the broad audience
- 10+ weeks of output on the wedge channel with no compounding
- CAC above LTV/2 and rising

Casey Winters: "The problem with frameworks is they tell you what to do, not why it works. Once conditions change, the framework breaks."

Do the diagnosis from first principles before you run the next channel experiment.

(Winters, 2023)

When to pivot the price

Pivot the price when users stay but don't pay, or when the price ceiling is lower than your cost structure.

Triggers:

- Trial-to-paid below 4% with D30 retention above 25%
- Net revenue retention below 80% (expansion revenue is not compensating for churn)
- Users describe the product as "a good value" but still cancel

Raise the price before you add features. Most early B2C founders are underpriced by 2x or 3x.

Honest assessment: should you keep going?

The three-question test

Before the capstone defense, answer these honestly:

1. Is the problem real? Do users report genuine pain without being prompted?
2. Is your solution the right one? Does your D30 retention reflect a product that delivers the value it promises?
3. Is the market reachable? Can you find more of these users at a cost below your LTV/3?

If the answer to all three is yes, the stall is a solvable problem. Diagnose the quadrant. Fix the primary cause.

If the answer to one is no, the stall is a signal, not a problem.

The uncomfortable conversation

Pieter Levels on bootstrapped persistence:

"Shipping is the easy part. Deciding to stop is the hardest skill."

The diagnostic quadrant does not tell you whether to keep going. It tells you what the obstacle is. Deciding whether that obstacle is worth clearing is a judgment call that belongs to you.

A clear diagnosis with a clear intervention is worth attempting. An unclear diagnosis with no evidence is a reason to pause.

(Levels, 2019)

Take-aways

1. MRR stalls have four primary causes: leak, ceiling, price, channel. Naming the right one first determines the correct fix.
2. The self-audit checklist produces five numbers. If you can't name all five, the diagnosis can't be trusted.
3. Pivot triggers for product, channel, and price each require specific evidence, not general frustration.
4. The diagnostic quadrant answers "what is the obstacle." The keep-going question belongs to you.
5. Friday: present your numbers, name your quadrant, defend your intervention.

What's next

- **Reading:** "Week 10 synthesis" at </c/b2c-10k-mrr-26au/readings/wk10> (required before Friday)
- **Lecture 2 (this week):** Past \$10k: what changes, the graduate-to-compound mindset, send-off
- **Section (Friday):** Stall diagnosis clinic. 15 minutes per student. Come with your dashboard live.
- **Capstone due Friday:** 5-minute defense with funnel, diagnosis, intervention, rollback